

Organ and Tissue Donation, Use and Retention

Summary Sets out requirements for both living and deceased organ and tissue donation. This includes requirements for obtaining consent from the donor and/or senior available next of kin; the requirement for a designated officer to provide written authorisation for the removal of organ/s and/or tissue after death; authorising ante-mortem procedures and the requirements and restrictions on living organ and tissue donation from adults and children.

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Organ and Tissue Donation, Use and Retention

Policy Statement

NSW Health provides organ and tissue donation and transplantation services in accordance with legislation, ensuring that consent is obtained before any procedure involving living or deceased donors.

NSW Health organisations must meet the requirements for:

- obtaining consent from the intended donor and/or senior available next of kin
- a designated officer providing written authorisation for ante-mortem procedures and the removal of organ/s and/or tissue after death; and
- restrictions on living organ and tissue donation from adults and children.

Summary of Policy Requirements

Local requirements and consent processes

NSW Health organisations must have local protocols and procedures in place which fit the requirements for both living and deceased organ and tissue donation.

This Policy Directive outlines the process for obtaining written consent, or consent by other manner prescribed, prior to the removal of organ and tissue for medical, scientific or therapeutic use (apart from diagnostic purposes) in line with the requirements of the *Human Tissue Act 1983* (NSW). Where the donor is deceased, and in the absence of their written consent, consent must be obtained from the senior available next of kin or their delegate.

The Policy Directive also outlines the process for obtaining consent and authorising ante-mortem procedures for potential tissue donors who lack capacity.

The process for obtaining consent and certification to remove regenerative tissue from a living child for the purpose of transplantation into a parent or sibling is also summarised.

Authorisation, objections and documentation

A designated officer must provide written authorisation for ante-mortem procedures and the removal of organ and tissue after death for use for donation and transplantation or for other therapeutic, medical or scientific purposes. Where a family objects to the donation of organs from a deceased patient, contrary to the known wishes of the donor, the requesting clinician must document the reasons for family objection and have this documentation signed by the designated officer.

This Policy Directive also summarises the process for assessing requests for the return of tissue to a patient and/or senior available next of kin.

This Policy Directive requires the use of NSW Health State Forms, including those for consent and authorisation for ante-mortem procedures, as well as consent and certification for the donation of organ(s) and tissue from a deceased patient/senior available next of kin, living adult and/or child.

Revision History

Version	Approved By	Amendment Notes
PD2025_039 October-2025	Deputy Secretary, Population and Public Health & Chief Health Officer	Updated Policy Directive to reflect a legislative change in the <i>Human Tissue Act 1983</i> (NSW) that allows a senior available next of kin to delegate their responsibility to authorise ante-mortem procedures to another person.
PD2024_022 July-2024	Deputy Secretary, Population and Public Health & Chief Health Officer	Updated Policy Directive to reflect a legislative change in the <i>Human Tissue Act 1983</i> (NSW) that allows ante-mortem procedures to be carried out for potential tissue donors.
PD2022_035 August-2022	Deputy Secretary, Population and Public Health & Chief Health Officer	Updated Policy Directive to reflect current practice and terminology.
PD2020_012 April-2020	Deputy Secretary, Population and Public Health and Chief Health Officer	Changes in relation to the time period for circulatory determination of death in the context of organ donation.
PD2016_001 February-2016	Deputy Secretary, Population and Public Health	New policy replacing sections 4.1 to 4.4 of PD2005_341. Human Tissue - Use/Retention of Including Organ Donation, Post Mortem and Coronial Matters and PD2012_014 Human Tissue – Consent for Donation of Regenerative Tissue by Young Children & Consent Form.

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1. Background

1.1. About this document

This Policy Directive sets out the requirements for both living and deceased organ and tissue donation. This includes:

- the requirements for obtaining consent from the intended donor and/or senior available next of kin
- the requirement for a designated officer to provide written authorisation for the removal of organ/s and/or tissue after death, and
- the requirements and restrictions on living organ and tissue donation from adults and children.

The NSW Organ and Tissue Donation Service (OTDS) *Standard Operating Procedure for the facilitation of organ and tissue donation in NSW* (Procedure) provides detail on the deceased donation process and is to be read in conjunction with this document.

This Policy Directive must also be read in conjunction with:

NSW Health Policy Directives, Guidelines and Manuals	
PD2024_036	<i>Coroners Cases and the Coroners Act 2009</i>
PD2025_040	<i>Designated Officer</i>
GL2025_014	<i>Organ Donation After Circulatory Death</i>
GL2021_004	<i>End of Life Care and Decision-Making</i>
GL2023_013	<i>Management of the Potential Organ and Tissue Donor following Neurological Determination of Death</i>
Consent Manual	<i>Consent to Medical and Healthcare Treatment Manual</i>
Privacy Manual	<i>Privacy Manual for Health Information</i>

NSW Health State Forms relevant to this Policy Directive	
SMR010516	<i>Documenting Family Objection to Organ/Tissue Donation Contrary to the Known Wishes of the Donor</i>
SMR010517	<i>Neurological Determination of Death (also called brain death)</i>
SMR010518	<i>Organ Donation - Circulatory Determination of Death</i>
SMR020020	<i>Consent to Regenerative Tissue Donation – Child Incapable of Understanding</i>
SMR020.235	<i>Consent and Authority for Removal of Organ(s) and Tissue After Death – Senior Available Next of Kin</i>
SMR020.236	<i>Consent and Authority to Ante-Mortem Procedures for Organ(s) and Tissue Donation After Death</i>

NSW Health State Forms relevant to this Policy Directive	
SMR020043	<i>Authorisation to Delegate Responsibilities of Senior Available Next of Kin for Organ and Tissue Donation Procedures</i>
SMR040044	<i>Authorisation to Delegate Responsibilities of Senior Available Next of Kin for Ante-mortem Procedures</i>
SMR020033	<i>Authorisation for the Release of Human Tissue to a Patient or Senior Available Next of Kin</i>
SMR020034	<i>Consent and Authority for the Retention and use of Tissue Removed or Expelled During Treatment of a Now Deceased Patient</i>
SMR020035	<i>Human Tissue Act 1983 Consent and Certification for the Donation of Tissue by a Living Adult</i>
SMR020036	<i>Human Tissue Act 1983 Consent and Certification for the Donation of Regenerative Tissue by a Living Child</i>

State Forms can be ordered from [Finsbury Green](#).

NSW Government Resource	
Guide to the Child Safe Standards	<i>Office of the Children's Guardian Guide to the Child Safe Standards</i>

1.2. Key definitions

Ante-mortem procedure	The following medical procedures, other than normothermic regional perfusion, carried out to determine, maintain or improve the viability of tissue for a relevant purpose: (a) the administration of medication, (b) the removal of blood and tissue for testing, (c) medical imaging and other diagnostic procedures, (d) blood transfusions for the purpose of improving organ viability (Section 27B <i>Human Tissue Act 1983</i> [NSW]).
Australian Organ Donor Register (AODR)	The national register where a person 16 years and over can record their organ and tissue donation decision for after their death.
Carer	A carer is an individual who provides unpaid, informal support and care to a family member, friend or neighbour who needs assistance because of disability, terminal illness, chronic illness and/or mental illness. The carer may or may not live with the individual.

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Child	A person who has not attained the age of 18 years and who is not married (Section 4 <i>Human Tissue Act 1983</i> [NSW]).
Child in care	A child in care is a child or young person who is less than 18 years old: • who is under the parental responsibility of the Minister administering the <i>Children and Young Persons (Care and Protection) Act 1998</i> (NSW), or • for whom the Secretary of the Department of Communities and Justice or a designated agency has the care responsibility, or • who is a protected person, or • who is the subject of an out-of-home care arrangement, or who is the subject of a sole parental responsibility order, or • who is otherwise in the care of a service provider. Parental responsibility in relation to a child or young person, means all the duties, powers, responsibilities and authority that, by law, parents have in relation to their children. (<i>Children and Young Persons (Care and Protection Act) 1988</i> [NSW]).
Child Safe Standards	Under the <i>Children's Guardian Act 2019</i> (NSW), NSW Health is required to implement the 10 Child Safe Standards where children and young people under the age of 18 years may be accessing or receiving NSW Health Services. The Child Safe Standards provide a framework for creating child safe organisations and are designed to drive cultural change to create, maintain and improve child safe practices.
Death	Death is an irreversible cessation of all function of the person's brain, or irreversible cessation of circulation of blood in the person's body (Section 33 <i>Human Tissue Act 1983</i> [NSW]).
De facto	Applies to individuals in de facto relationships with another person (whether of the same sex or a different sex), if • they have a relationship as a couple living together for at least 2 years, and • they are not married to one another or related by family A de facto relationship can exist even if one of the persons is legally married to someone else or in a registered relationship with someone else.

Delegate of the senior available next of kin	Is a person who has been authorised in writing by the senior available next of kin to exercise his/her functions (Section 5A or section 27DA <i>Human Tissue Act 1983</i> [NSW]).
Designated officer	A designated officer is: • In relation to a public hospital, a person appointed in writing by the governing body of a hospital to be a designated officer for the hospital, or • In relation to a private hospital, a person appointed in writing by the governing body (licensee) of the private hospital, or • In relation to a forensic institution, a person appointed in writing by the governing body of a forensic institution to be a designated officer for the forensic institution (Section 5(1) <i>Human Tissue Act 1983</i> [NSW]) Also, refer to NSW Health Policy Directive <i>Designated Officer</i> (PD2025_040).
Designated specialist	A medical practitioner who holds a specialist registration in one or more of the following specialties may be appointed as a designated specialist: • Anaesthesia • Emergency medicine • Intensive care medicine • Obstetrics and gynaecology • Physician • Surgery (Section 5(1)(b) <i>Human Tissue Act 1983</i> [NSW]) • or a medical practitioner appointed by the Chief Health Officer or a Local Health District Chief Executive. (NSW Health Policy and Procedure Manual Public Health Delegations Manual Section 4.2).
DonateLife network	The Australian Government provides funding to state and territory governments to maintain an organ and tissue service referred to as the DonateLife network. This is the NSW Organ and Tissue Donation Service (OTDS).
Donation Specialist Coordinator (DSC)	An appropriately trained nursing professional who is responsible for the coordination and facilitation of the organ and tissue

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	donation and retrieval process state-wide. Donation specialist coordinators are based at the NSW Organ and Tissue Donation Service.
Donation Specialist Nurse (DSN)	An appropriately trained nursing professional who is responsible: • for raising awareness of, and providing clinical and educational services on organ and tissue donation for all Local Health Districts (districts)/Specialty Health Networks (networks) medical, nursing and allied health staff • providing quality and safety feedback to clinicians regarding identification and assessment of potential organ donors and the donation process • being actively involved in all end of life conversations where organ and tissue donation is considered and the Family Donation Conversation, ensuring that all elements of the Organ and Tissue Authority's <i>Best Practice Guideline for Offering Organ and Tissue Donation in Australia (Edition 2, April 2021)</i>, for offering organ and tissue donation are met, in particular, the collaborative approach • donor family support and follow up • for auditing deaths and organ donation referral practices across districts/networks.
Donation Specialist Medical (DSM)	An appropriately trained medical professional who is responsible for all processes to optimise organ and tissue donation for transplantation in their hospital and, where applicable, specified local referral hospitals.
Electronic donor record (EDR)	Captures donor referral data, medical-social history for potential donors, family consent information and provides a real time electronic system for the offering and allocation of organs and tissues across Australia and New Zealand. The information may be disclosed and only within the organ and tissue donation network across Australia (the DonateLife network), to transplant units, eye and tissue banks and sometimes to the organ donation network in New Zealand, where relevant.

Enduring guardian	The person an individual legally appoints to make decisions about their health and lifestyle in the event they cannot make these decisions for themselves (Part 2 <i>Guardianship Act 1987</i> [NSW]). For further guidance refer to the NSW Health Policy and Procedure Manual <i>Consent to Medical and Healthcare Treatment Manual</i> (Consent Manual).
Family donation conversation (FDC) training	The Family Donation Conversation training consists of core and practical training for health professionals working in deceased organ and tissue donation. For more information visit the Australian Organ and Tissue Authority.
NSW Organ and Tissue Donation Service (OTDS)	The NSW Organ and Tissue Donation Service (OTDS) develops, coordinates and maintains clinical and operational protocols for state-wide organ and tissue donation in NSW. Coordination of deceased organ and tissue donation at district and network level is managed by Donation Specialist staff employed in selected hospitals. Coordination of tissue-only donation is facilitated by the tissue donor coordination team from the NSW Tissue Bank. Further information is available from the NSW Organ and Tissue Donation Service.
NSW Tissue Bank	The NSW Tissue Bank is part of the NSW Organ and Tissue Donation Service (OTDS) and is responsible for the identification, assessment and consent of donors, procurement and manufacture of tissue, research and development.
Person responsible	If a person is not capable of consenting to their own treatment, the health practitioner must seek consent from their 'person responsible'. This is required and there is a hierarchy of people who can be the person responsible, in order of priority: • Guardian – an appointed guardian (or enduring guardian) who has been given the right to consent to medical and dental treatments, or • Spouse or partner – if there is no guardian, a spouse, de-facto spouse or partner (including same sex partner) where there is a close continuing relationship, as long as they are not a person under guardianship, or • Carer – if there is no spouse or partner, an unpaid carer who provides or arranges for domestic support on a regular basis, or • Relative or friend – if there is no carer, a friend or relative who has a close personal relationship, frequent personal

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	contact and a personal interest in the person's welfare, on an unpaid basis. The person responsible for a child (a person less than 18 years) is the person having parental responsibility for the child. However, if the child is in the care of the Minister responsible for the <i>Children and Young Persons (Care and Protection) Act 1998</i> (NSW), then the Minister is the person responsible, or, if the child is in the care of the Secretary for the NSW Department of Communities and Justice, then the Secretary is the person responsible (Section 33A <i>Guardianship Act 1987</i> [NSW]).
Principal Care Officer	In relation to a child in care of the State, the Principal Care Officer is the Secretary under the <i>Children and Young Persons (Care and Protection) Act 1998</i> (NSW) [Section 4(8) <i>Human Tissue Act 1983</i> (NSW)] or their delegate.
Requesting health professional	An appropriately qualified health professional who has experience in conducting sensitive conversations with families and has attended Family Donation Conversation training or its equivalent. The requesting health professional's role is to collaborate with the Donation Specialist and empower the family to make a fully informed and enduring decision (the collaborative approach). They will discuss the possibility of donation with the family and inform them of the individual's documented decision where recorded.
Senior available next of kin	As per Section 4 of the <i>Human Tissue Act 1983</i> (NSW), a senior available next of kin is: • In relation to a child who is living: ○ a parent of the child, or ○ if a parent is not available, a person who is a guardian of the child. • In relation to a child who is a potential tissue donor or a deceased child: ○ a parent of the child, or ○ if a parent of the child is not available, a sibling of the child who is 18 years of age or over, or ○ if none of the above is available, a person who was a guardian of the child immediately before the death of the child. Where the child is in the care of the State, specific provisions for consent to organ and tissue donation apply (see Section 2.8).

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	In relation to any other potential tissue donor or deceased person, a senior available next of kin is: • a person's spouse (which can include a de facto spouse and same sex partner), or • if there is no spouse or the spouse is not available, a child of the potential tissue donor or deceased person who is at least 18 years of age, or • if no person referred to above is available, a parent of the potential tissue donor or deceased person, or • if no person referred to above is available, a brother or sister of the potential tissue donor or deceased person who has attained the age of 18. The potential tissue donor is a person whom an ante-mortem procedure is to be carried out.
Tissue	As per Section 4 of the <i>Human Tissue Act 1983</i> (NSW), tissue refers to an organ or part of a human body and a substance extracted from, or from a part of, the human body. There are 2 types of tissue: • Regenerative tissue is tissue that, after injury or removal, is replaced in the body of a living person by natural processes of growth or repair (such as bone marrow). • Non-regenerative tissue is tissue other than regenerative tissue. As per Section 34 <i>Human Tissue Act 1983</i> (NSW), tissue blocks and slides may be retained without specific consent when removed for the purpose of carrying out tests. Refer to NSW Health Guideline <i>Use of Human Tissue for Research</i> (GL2023_008) for specific requirements regarding donation of organs and/or tissue for other medical and scientific use.
Tissue donor coordinator (TDC)	A Tissue Bank specialist scientist or nurse responsible for the coordination and facilitation of tissue donation process from living and deceased donors.

1.3. Legal and legislative framework

This Policy Directive is underpinned by the requirements of:

- The [Human Tissue Act 1983](#) (NSW)
- The [Human Tissue Regulation 2020](#) (NSW)
- The [Health Records and Information Privacy Act 2002](#) (NSW)
- The [Public Health Regulation 2022](#) (NSW)
- The [Coroners Act 2009](#) (NSW)
- The [Guardianship Act 1987](#) (NSW)
- The [Children's Guardian Act 2019](#) (NSW).

2. Consent for deceased organ and tissue donation

A valid consent is essential for deceased organ and tissue donation (refer to the NSW Health Policy and Procedure Manual *Consent to Medical and Healthcare Treatment Manual* ([Consent Manual](#))). For deceased organ and tissue retrieval to proceed, consent must have been given by either the deceased person during their lifetime or by their senior available next of kin. The hospital's designated officer must also have granted authorisation to remove the organ/s and/or tissue.

Part 4 of the *Human Tissue Act 1983* (NSW) defines who can consent and who can authorise the removal of organs and tissue from a deceased person. Section 25 of the *Human Tissue Act 1983* (NSW) outlines specific requirements that also apply to the removal of organs and tissue from deceased persons whose deaths are reportable to the coroner.

In NSW the Organ and Tissue Authority's *Best Practice Guideline for Offering Organ and Tissue Donation in Australia* ([Best Practice Guideline](#)) is used when obtaining consent from the senior available next of kin for organ and tissue donation. This also ensures that a Donation Specialist participates in the Family Donation Conversation.

Consent must be in writing or by other manner prescribed (as described below).

Certain ante-mortem procedures can also be carried out under the *Human Tissue Act 1983* (NSW) where consent is obtained and certain safeguards are met (see [Section 3](#)).

2.1. Written consent of donor

If the deceased person gave written consent during their lifetime for the removal of their organs and/or tissue for transplantation or another therapeutic, medical or scientific purpose, and the consent had not been revoked, the designated officer may authorise removal of the organs and/or tissue in accordance with the deceased's consent. For example, such written consent may be in the form of registration on the Australian Organ Donor Register or an advance care directive. For more information on advance care directives, refer to NSW Health's [Making an Advance Care Directive](#).

If the deceased person gave written consent for donation, there is no legal requirement under the *Human Tissue Act 1983* (NSW) to obtain consent from the senior available next of kin. However, in practice, consent will be sought from the senior available next of kin as donation will not proceed against the family's wishes. Where there is no senior available next of kin and the deceased person has given their written consent during their lifetime, the designated officer can authorise the removal of organs and/or tissue.

For a deceased child, other than a child in the care of the State, the written consent of the senior available next of kin is required.

For a child in the care of the State, written consent must be obtained from the Principal Care Officer of the agency that has responsibility for the child as outlined in Section 23A and 24A of the *Human Tissue Act 1983* (NSW) (see [Section 2.8](#)).

2.2. Determining the deceased person's most recent decision

Donation specialist and tissue donor coordinators of the NSW Organ and Tissue Donation Service must access the Australian Organ Donor Register to determine if the deceased person has previously registered a decision regarding organ or tissue donation.

This information must be obtained prior to the family donation conversation so that the family can be advised of the deceased person's decision. This information must also be available to the designated officer.

Before issuing their written authority for the removal of organs and/or tissue, the designated officer must be satisfied that the deceased person had not, when living:

- expressed an objection to organ and tissue donation
- expressed an objection to the removal of organs and/or tissue from the person's body after the person's death.
- If the person had expressed such an objection, based on the most recent views expressed by the deceased, the person no longer had an objection to the removal of organs and/or tissue from the person's body.

This requirement recognises the situation where a person may have registered a refusal on a donation register, subsequently changed their decision, but did not re-register their donation decision. This allows a designated officer to consider information from the senior available next of kin, relatives, friends and social media that the deceased person has subsequently changed their mind and has indicated that they would like to donate. NSW Health Policy Directive *Designated Officer* ([PD2025_040](#)) further describes the role and responsibilities of the designated officer when authorising the removal and use of organs and tissue.

If there is no new information indicating that the deceased person had subsequently expressed an objection to organ and tissue donation after their death, then donation can proceed with a valid consent from the senior available next of kin (see [Section 2.3](#)) or the delegate of the senior available next of kin and authorisation from the designated officer.

Once consent to the removal of organs and tissue for the purpose of donation has been obtained, as required by Part 4 of the *Human Tissue Act 1983* [NSW] (see Sections [2.1](#), [2.2](#), [2.3](#)), a designated officer may provide written authorisation for the removal of organs and

tissue from the deceased person as specified in the consent (Section 23(1) *Human Tissue Act 1983* [NSW]).

2.3. Consent from the senior available next of kin

After making reasonable inquiries, if a designated officer is not satisfied that the deceased person gave their written consent during their lifetime to the removal of organs and tissue for the purpose of transplantation, written consent must be obtained from a senior available next of kin. The State Form *Consent and Authority for Removal of Tissue After Death* (SMR020030) is to be used.

Only then is the designated officer able to authorise the removal of organs and tissue.

Persons appointed as Guardians under the *Guardianship Act 1987* (NSW), persons who hold a 'Power of Attorney', persons identified as a 'person responsible' or a 'person to notify' in a hospital or medical record are not able to give consent to deceased donation for another person under the *Human Tissue Act 1983* (NSW). They can only give consent if they are also the senior available next of kin or the senior available next of kin has delegated their authority in writing to them.

Where the deceased is a child, consent for removal of organs and tissue must be obtained from a senior available next of kin. However, in the case of a child in the care of the State, written consent must be obtained from the Principal Care Officer of the agency that has responsibility for the child (see [Section 2.8](#)).

2.3.1. Cultural considerations in defining next of kin

It is important to recognise that the Aboriginal and Torres Strait Islander family system has an extended family structure which can differ from the immediate family structure which is common in western society. This means the child rearing responsibilities extend beyond the immediate family group and may include aunts, uncles, cousins and grandparents. The family and those with cultural authority for the child are the ones who are responsible for deciding who is considered kin to a child¹ (this may differ from the legal definition of senior available next of kin under the *Human Tissue Act 1983* [NSW]). This may also apply in other cultures where the person considered to be kin may differ from the legal definition of senior available next of kin under the *Human Tissue Act 1983* (NSW). In these situations, refer to Section 2.4, regarding the delegate of the senior available next of kin.

2.3.2. Consent by other manner prescribed (audio and audio-visual)

Section 23(3)(b) and Section 24(3) of the *Human Tissue Act 1983* and Clause 8 of the *Human Tissue Regulation 2020* (NSW) permit consent for deceased organ and tissue donation to be obtained from a senior available next of kin using audio or audio-visual recording (other manner prescribed). If this form of consent is used, the following provisions must be met:

¹ Queensland Government [The meaning of family in Aboriginal and Torres Strait Islander cultures](#)

- consent must be obtained from the senior available next of kin to conduct a recording from the person/s being recorded, and
- the recording must comply with the mandatory requirements for obtaining a valid consent to organ and tissue donation.

The following elements of the consent discussion with the senior available next of kin must be recorded:

- the date and time of the recording
- the name and designation of the health professional obtaining consent
- the name, age (or date of birth) and address of the person giving consent and their relationship to the deceased
- a statement that the person giving consent is the senior available next of kin or delegate of the senior next of kin
- information on the organs and tissue to which consent applies and their potential uses
- any other specific requirements of the consent, such as use for research, and
- an offer for the family to ask questions.

The audio or audio-visual recording is the legal instrument of consent and a copy must be archived with the patient's medical record and uploaded to the electronic donor record or relevant donor file.

2.4. Consent from the delegate of the senior available next of kin

In some cultures and communities, it is usual for responsibilities relating to death to be undertaken by someone other than the person who would be legally defined as the senior available next of kin under the *Human Tissue Act 1983* (NSW).

In these circumstances it is important for the treating team to engage with Aboriginal Health Workers, Aboriginal Health Practitioners and Multicultural Health Workers to ensure the appropriate people have been consulted.

Section 5A and 27DA of the *Human Tissue Act 1983* (NSW) provides an option for the senior available next of kin to delegate their authority in writing to another person, who may then assume the functions of the senior available next of kin. It is essential that this delegation is made with the consent of the senior available next of kin and that they understand what granting such an authority to another person will mean.

The State Form *Authorisation to Delegate Responsibilities of Senior Available Next of Kin for Ante-Mortem Procedures* (SMR040.044) and/or *State Form Authorisation to Delegate Responsibilities of Senior Available Next of Kin for Organ and Tissue Donation Procedures* (SMR020.043) must be attached to the consent documentation and provided to the designated officer.

2.5. Family Donation Conversation with the deceased donor's family / senior available next of kin

The treating health professional must contact the local Donation Specialist or NSW Organ and Tissue Donation Service prior to the commencement of discussions with the family regarding end of life (see NSW Health Guideline *End of Life Care and Decision-Making* [GL2021_004]) and prior to the Family Donation Conversation, to determine if the deceased person had registered a donation decision.

This information is shared with the family during the Family Donation Conversation for consideration during their decision-making process. For tissue-only donation, discussion with family may occur via telephone contact by the Tissue Donor Coordinator at the NSW Tissue Bank.

The Family Donation Conversation must be handled with great care and sensitivity and in accordance with the Organ and Tissue Authority's *Best Practice Guideline for Offering Organ and Tissue Donation in Australia* ([Best Practice Guideline](#)).

2.5.1. Cultural considerations in family discussions

When discussing organ and tissue donation with the family and those close to the deceased, it is important for health professionals to recognise that different cultural groups will have differing ideas and beliefs that influence their views about donation. For example, in Aboriginal culture it is considered taboo to mention (or in some cases write) the name of a deceased person given a belief that if the deceased person's name is mentioned, the spirit is called back to this world.

In addition, it is considered culturally inappropriate for a non-Indigenous NSW Health staff member to contact and inform the senior available next of kin of a person's passing. This breach of cultural protocol may cause significant distress for Aboriginal and Torres Strait Islander families connected to the person who has passed^{2,3}.

There are several health resources and factsheets to assist Aboriginal and Torres Strait Islander families when making decisions relating to organ and tissue donation.

Health professionals are to seek assistance and support from the local Aboriginal Hospital Liaison Officer where available.

Health professionals should also note that providing accessible information for families with English as a second language is essential to support the decision-making process.

Relevant resources and fact sheets to support Aboriginal and Torres Strait Islander families, as well as families from culturally and linguistically diverse backgrounds, can be found on the Australian Government [Organ and Tissue Authority](#).

² *Sad News, Sorry Business: Guidelines for caring for Aboriginal and Torres Strait Islander people through death and dying (version 2)* Published by the State of Queensland (Queensland Health), December 2015

³ NSW Health Guideline *Communicating Positively: A Guide to Appropriate Aboriginal Terminology* (GL2019_008)

2.6. Managing family objections

If the immediate family and/or the extended family and/or those close to the deceased person object to donation and the deceased had expressed a written or verbal consent in their lifetime to donate, the basis of these objections must be explored.

The *Human Tissue Act 1983* (NSW) makes no provision for families to override prior consent for organ and tissue donation. However, for prudential and compassionate reasons, family agreement is generally sought.

If there is dissent amongst the family about proceeding with donation, the benefits of carrying out the donation must be weighed against the distress and resentment that may result if donation proceeds in the face of strong objection from some family members.

The decision not to proceed with organ and tissue donation in this context depends on the presence of strong and sustained family objection in spite of appropriate information provision and time to reflect.

If the objection to donation is contrary to the documented decision of the deceased person ('override'), the health professional conducting the Family Donation Conversation and the designated officer must complete the State Form *Documenting Family Objection to Organ/Tissue Donation Contrary to the Known Wishes of the Donor* (SMR010516). The completed form must be signed by the senior available next of kin, the designated officer and the requesting health professional. This form must be uploaded to the person's medical record, as well as to the referral log by local donation staff or a scanned copy can be sent to the NSW Donation Specialist Coordinator (DSC).

The Donation Specialist or requesting health professional who led the Family Donation Conversation must contact the NSW Organ and Tissue Donation Service State Medical Director at the time of the override to discuss the circumstances of the family decision.

For tissue-only donation, reasons for objection are documented within the NSW Tissue Bank Deceased Referral Log.

2.7. Coronial consent

Refer to NSW Health Policy Directive *Coroners Cases and the Coroners Act 2009* ([PD2024_036](#)) for more information on the role and responsibility of the coroner.

If a coroner has jurisdiction to hold an inquest into the death of a person under the Coroners Act 2009 [NSW] (for example the death is reportable to the coroner), the designated officer or Principal Care Officer must not authorise the removal of tissue from the person's body unless a coroner has given consent to the removal of the tissue (see Section 25 *Human Tissue Act* [NSW]).

The *Human Tissue Act 1983* (NSW) allows for coronial consent to be given before a person's death if the coroner reasonably believes there will be a coronial inquest into the person's death under the *Coroners Act 2009* (NSW).

It is the treating medical team's responsibility to determine if the death is reportable to the coroner, utilising the relevant State Form *Coronial Checklist* (SMR01513). The treating team is responsible for reporting the death to the coroner. The Donation Specialist or Tissue Donor Coordinator is to liaise directly with the Forensic Pathologist and the coroner.

2.8. Children in the care of the State

A child is in the care of the State if the Minister administering or the Secretary under the *Children and Young Persons (Care and Protection) Act 1998* (NSW) has sole parental responsibility in respect of the child (or young person).

Section 34A of the *Human Tissue Act 1983* (NSW) prohibits the removal and use of organs and tissue from a deceased child in the care of the State to be used for any medical, therapeutic or scientific purposes other than that for transplantation to the body of a living person.

2.8.1. Consent responsibilities of the Principal Care Officer

The Principal Care Officer has responsibility for consent for organ and tissue donation for transplantation, as per Sections 23A(b) and 24A of the *Human Tissue Act 1983* (NSW).

Before deciding whether or not to give consent for the donation and removal of organs and tissue from the deceased child's body, the Principal Care Officer must make reasonable efforts to contact persons who have been significant in the child's life and who the Principal Care Officer considers to be appropriate to assist in the decision-making process. This may include:

- birth parents
- foster parents
- extended family
- if the child is Aboriginal or Torres Strait Islander, appropriate persons from the child's immediate and/or extended family and/or Aboriginal and/or Torres Strait Islander community, and
- other persons considered relevant by the Principal Care Officer.

The Principal Care Officer is to determine whose approval is required and may determine that more than one person's approval is necessary. The Principal Care Officer must record the actions that they have taken and the details of any parties they have contacted, or attempted to contact, as this information may be required by the designated officer prior to granting their authorisation.

The Principal Care Officer must not give consent unless all relevant parties have been consulted and have given their approval to the donation, as per Section 24B of the *Human Tissue Act 1983* (NSW).

The complexity of the consent and authorisation process requires conversation between the Principal Care Officer, the coroner and the designated officer to facilitate a timely process and ensure that all legal authorisations are obtained.

The designated officer must have access to the written consent of the Principal Care Officer and be satisfied that the child did not express an objection during his or her lifetime about removal and donation of their organs and tissue after death.

Under Section 25 of the *Human Tissue Act 1983* (NSW), the consent of the coroner must also be obtained prior to the removal of organs and tissue from the deceased child. The

designated officer may authorise removal of organs and tissue from a deceased child once the Principal Care Officer has given their written consent and consent from the coroner has been obtained.

2.9. Consent for other therapeutic, medical or scientific purposes (research purposes included)

Organs and tissue can only be used for research purposes if the research has been reviewed and approved by a Human Research Ethics Committee in accordance with the National Health and Medical Research Council [National Statement on Ethical Conduct in Human Research](#) (2007).

Organs and tissue may only be removed from a deceased person for research purposes where written consent was obtained either by the person during their lifetime (or by their parent/guardian if they were a child) or after death by the senior available next of kin and with the written authorisation of a designated officer. For specific consent requirements, refer to the NSW Health Guideline *Use of Human Tissue for Research* ([GL2023_008](#)).

Where organs are deemed unsuitable for transplantation but suitable for research, the retrieving surgeons or transplant centre must inform the Donation Specialist Coordinator.

In no circumstances are organs and tissue to be removed for research purposes from the body of a deceased child in the care of the State, Section 34A of the *Human Tissue Act 1983* (NSW) prohibits such removal and use (see [Section 2.8](#)).

3. Deceased organ and tissue donation protocol

The NSW Organ and Tissue Donation Service Standard Operating Procedure for the Facilitation of Organ and Tissue Donation in NSW ([Procedure](#)) applies to both donation after neurological determination of death and donation after circulatory determination of death pathways.

Refer to the NSW Organ and Tissue Donation Service Standard Operating Procedure for Facilitation of Organ and Tissue Donation in NSW for specific guidance on blood specimens and tissue typing requirements.

3.1. Identification and notification of potential deceased organ and tissue donors

Local health districts must have systems in place to identify and refer all potential deceased organ and tissue donors and to provide opportunities for families to support donation for transplantation.

3.1.1. Organ donation

Hospitals are to have local protocols in place to ensure that all patients who are ventilated (including invasive and non-invasive) in an Emergency Department (ED) or Intensive Care Unit (ICU) for whom end of life discussions are being held, are referred to the NSW Organ and Tissue Donation Service for the consideration of organ and tissue donation.

Local hospital protocols are to be in place between EDs and ICUs for the ongoing care of patients who are being considered for organ and tissue donation.

3.1.2. Classification of the organ and tissue donor for billing purposes

Private patients must be converted to public patients once consent for organ and tissue donation has been obtained. All subsequent investigations must not be charged to a private patient's account. Refer to the NSW Health [Financial Class \(EDW\) Masterlist](#) for the current Financial Class values to be used for organ donors.

All relevant paperwork must be labelled or stamped "FOR ORGAN DONATION, NOT FOR BILLING".

3.1.3. Tissue-only donation

A potential deceased tissue donor is a deceased person for whom tissue retrieval is possible for up to 36 hours after death depending on the tissue type.

All deaths (including coronial cases) occurring within or upon arrival at a hospital are to be notified as a matter of urgency to the NSW Tissue Bank. This referral must be in accordance with local health district and specialty health network protocols for electronic notification via the electronic medical record (EMR).

This is an automatic referral to the [NSW Tissue Bank](#), except in rural areas, which are exempt from mandatory notification due to the logistics of tissue retrieval. Eye donation is available in some rural areas and in these instances coordination and consent is managed by a Tissue Donor Coordinator.

The minimum information for notification must include:

- name and date of birth of the deceased
- time and date of death, and
- medical record number, ward and hospital.

3.2. Referral to the Donation Specialist and Tissue Donor Coordinator

Contact details for the local Donation Specialist and the NSW Tissue Bank must be available throughout the hospital.

On referral, the Donation Specialist and Tissue Donor Coordinator will require the following information:

- name, address and date of birth of the potential donor
- reason for hospital admission
- cause of death or reason for proposed withdrawal of cardio-respiratory support
- information on planned discussions regarding end of life
- current haemodynamics and ventilator settings

- current blood results (biochemistry and haematology)
- past medical history, including GP summary or contact details, if available, and
- details of senior available next of kin for the Family Donation Conversation (see [Section 2.5](#)).

This information will be used by the Donation Specialist and Tissue Donor Coordinators to determine preliminary medical suitability of potential donors.

3.3. Determination of death

As set out in Section 33 of the *Human Tissue Act 1983* (NSW), for the purposes of the law in NSW, a person has died when the following has occurred:

- irreversible cessation of all function of a person's brain (also known as neurological death), or
- irreversible cessation of circulation of blood in a person's body (also known as circulatory death).

3.3.1. Neurological determination of death

The Australian and New Zealand Intensive Care Society (ANZICS) has established the criteria for neurological determination of death in Section 1.2 'Neurological determination of death' in *The ANZICS Statement on Death and Organ Donation* edition 4.1, 2021 ([ANZICS Statement](#)).

For neurological determination of death to be conducted, there must be definite clinical or neuroimaging evidence of acute brain pathology consistent with deterioration to permanent loss of all neurological function. In cases of hypoxic-ischaemic encephalopathy, clinical history alone may provide sufficient explanation of the acute brain pathology and not require neuroimaging prior to neurological determination of death by clinical examination. Regardless of imaging, clinical examination is a requirement for neurological determination of death.

When using clinical examination to determine death there must be a minimum of 4 hours observation and the following preconditions met:

- normothermia – temperature is $\geq 35^{\circ}\text{C}$
- normotension (such as Mean Arterial Pressure (MAP) > 60 in an adult)
- exclusion of sedative medications
- absence of severe electrolyte, metabolic or endocrine disturbances
- absence of acute liver failure or decompensated chronic liver disease
- absence of neuromuscular-blocking drugs
- ability to adequately examine the brainstem reflexes
- cervical level spinal cord injury may also preclude reliable sensory and motor assessment
- ability to perform apnoea testing.

If any of these preconditions cannot be met, brain perfusion studies are to be used to inform neurological determination of death.

Where neurological criteria are used, two (2) medical practitioners who have practiced medicine for at least 5 years in the previous 8 years must independently certify that death has occurred.

One of the medical practitioners must be a designated specialist appointed by the governing body of the hospital.

Neither medical practitioner can be associated with the retrieval process, nor responsible for the care of the intended recipient.

The State Form *Neurological Determination of Death* (also called brain death) (SMR010517) must be completed by both medical practitioners.

3.3.2. Circulatory determination of death

Circulatory determination of death must be certified by a medical practitioner in accordance with standard hospital procedures. The NSW Health Guideline *Organ Donation After Circulatory Death* ([GL2025_014](#)) describes the necessary requirements for health facilities to undertake donation after circulatory determination of death in NSW.

Section 1.3 'Circulatory determination of death' of ANZICS *The Statement on Death and Organ Donation* edition 4.1, 2021 ([ANZICS Statement](#)) requires that death be determined when the following features are present:

- absence of spontaneous movement, breathing and circulation
- absence of circulation as evidenced by absent arterial pulsatility for 5 minutes, using intra-arterial pressure monitoring or electrical asystole are to be observed for 5 minutes on the electrocardiogram, and confirmed by clinical examination
- death is confirmed when there is absent heart sounds and/or central pulse.

The medical practitioner responsible for determining death must document the time and date of the determination of death along with his or her name, designation and signature. The State Form *Organ Donation - Circulatory determination of death* (SMR010518) is to be completed.

3.4. Ante-mortem procedures

The following ante-mortem procedures are allowed, other than normothermic regional perfusion, to be carried out to determine, maintain or improve the viability of organs and tissues for transplantation where consent and authorisation have been obtained and documented:

- the administration of medication
- the removal of blood and tissue for testing
- medical imaging and other diagnostic procedures
- blood transfusions for the purpose of improving organ viability.

3.4.1. Consent to carry out ante-mortem procedures

A potential tissue donor can provide written consent to ante-mortem procedures.

Where the potential tissue donor lacks capacity to provide consent for ante-mortem procedures, then a senior available next of kin of the potential tissue donor can provide consent.

The senior available next of kin must not provide consent if there is reason to believe that the potential tissue donor has expressed an objection to the carrying out of an ante-mortem procedure on themselves.

A senior available next of kin can delegate, in writing, their responsibility to consent to ante-mortem procedures to another person.

In the event where the potential tissue donor has not provided consent to ante-mortem procedures and there is no known senior available next of kin of the potential tissue donor, then a designated officer must, in order to authorise ante-mortem interventions, be satisfied that:

- the potential tissue donor has, during the potential tissue donor's lifetime, provided consent in writing to the removal of tissue for a *relevant purpose, and
- the consent (for removal of tissue) has not been revoked, and
- the potential tissue donor has not expressed an objection to the carrying out of an ante-mortem procedure on the potential tissue donor.

*As per the *Human Tissue Act 1983* (NSW), relevant purpose means the expected transplantation of tissue, excluding gametes, from a potential tissue donor's body, after the potential tissue donor's death, to the body of another living person.

3.4.2. Authorisation of ante-mortem procedures

For ante-mortem procedures to be authorised, the designated officer must be reasonably satisfied that:

1. consent and authorisation to remove tissue after the death of the potential tissue donor is complete and valid or will be given, and
2. a *prescribed practitioner has certified in writing that they are reasonably satisfied the death of the potential tissue donor is imminently expected, and the carrying out of an ante-mortem procedure will not:
 - i. hasten the death, or
 - ii. cause more than minimal harm, or
 - iii. cause undue risk to the potential tissue donor.

*A prescribed practitioner is a designated specialist or if they are not available, an experienced medical practitioner who is not involved in transplantation procedures, or the care of the potential tissue donor or the recipient of tissue.

However, an ante-mortem procedure must not be carried out if:

- a potential tissue donor, has during their lifetime, expressed an objection to the carrying out of ante-mortem procedures on themselves
- a person responsible has consented to treatment under the *Guardianship Act 1987* (NSW) and the treatment is incompatible with the ante-mortem procedure
- it is reasonably likely to interfere with the functions of the coroner under the *Coroners Act 2009* (NSW).

3.5. Donor management

For specific clinical management guidelines for the potential donor after neurological determination of death, refer to the NSW Health Guideline *Management of the Potential Organ and Tissue Donor following Neurological Determination of Death* ([GL2023_013](#)).

For potential donors for whom donation after circulatory determination of death is being considered refer to NSW Health Guideline *Organ Donation After Circulatory Death* ([GL2025_014](#)).

3.6. Specimens

Blood specimens for serological testing and tissue typing are obtained as part of the organ and tissue donation process. The NSW Organ and Tissue Donation Service will coordinate the collection and transport of specimens, and will provide specific guidance to local health districts and speciality health network staff where a Donation Specialist is not available to manage the process.

3.6.1. Blood typing and sub-typing requirements

The Donation Specialist will need to obtain a National Association of Testing Authority (NATA) copy of the blood group (ABO) from the hospital pathology laboratory for tissue typing. If a blood group has not been previously obtained during this hospital admission, the Donation Specialist will require an immediate blood group.

Potential donors who have an ABO of either A or AB may require sub-typing to facilitate appropriate matching of their organs with potential recipients. If the hospital caring for the potential donor is unable to perform this subtyping, the Donation Specialist will arrange for the testing to be performed by the Australian Red Cross Lifeblood Service Red Cell Reference Laboratory.

3.6.2. Blood specimens for tissue typing and serological testing

All deceased donors require serology and Nucleic Acid Testing (NAT) to be performed.

Organ retrieval surgery may proceed in the absence of serology results, at the direction of the NSW Organ and Tissue Donation Service state medical director. However, no organs may be transplanted until the serology results have returned and been reviewed by the transplanting team.

NAT is performed on all organ and tissue donors with results available prior to or post retrieval surgery, dependent upon the risk profile of the deceased person. For further

information, refer to the Transplantation Society of Australia and New Zealand (TSANZ) *Clinical Guidelines for Organ Transplantation from Deceased Donors* ([Clinical Guidelines](#)).

Blood specimens can be drawn for serological testing and tissue typing as soon as consent (verbal consent is acceptable) has been obtained. If verbal consent is provided, this must be documented in the patient's medical record prior to blood collection and the written consent completed as soon as possible.

3.6.3. Blood specimen collection

The Donation Specialist and/or the bedside nurse are responsible for collection and packaging of the blood specimens for organ donors (instructions can be found in the organ and tissue donation blood specimen boxes).

If the facility does not have access to blood specimen boxes for organ and tissue donation, staff must contact the NSW Organ and Tissue Donation Service.

3.7. Organisation of retrieval surgery

The Donation Specialist Coordinator will liaise with the retrieval and transplant teams in collaboration with the Donation Specialist Nurse of the local hospital Intensive Care Unit and operating theatre, hospital executive and administration, to coordinate an operating theatre time. For more information, see the NSW Health Guideline *NSW Emergency Surgery Guidelines and Principles for Improvement* ([GL2021_007](#)).

The donor family are to be offered the opportunity of viewing the deceased after the retrieval surgery. The Donation Specialist staff or appropriate local health professionals will facilitate this viewing with the family. The location of the viewing will be dependent upon local hospital protocols, staffing and resources.

3.7.1. Tissue-only retrieval

In a tissue-only donation, tissue retrieval is coordinated to occur within 36 hours of death and is performed by appointed non-medical staff specialising in the retrieval procedure or by medical practitioners. Using a clean technique, eye-only retrieval can occur in hospitals, mortuaries, nursing homes and funeral homes. Musculoskeletal, skin and cardiovascular retrieval usually occurs in a mortuary or operating theatre using aseptic surgical technique.

4. Procedure for obtaining consent from living persons

This Section sets out the requirements for obtaining consent for the removal, collection, use and retention of organs and tissue removed from living persons.

Living donation programs fall into 2 categories:

- organs and tissue removed during medical, dental or surgical treatment
- living donation for transplantation.

A person has decision making capacity if they can:

- understand the facts involved
- understand the main choices
- weigh up the consequences of the choices
- communicate their decision.

Legally, adults are presumed to have capacity to consent to or refuse medical treatment unless otherwise indicated.

In all cases, consent must follow the normal consent procedures outlined in the NSW Health Policy and Procedure Manual *Consent to Medical and Healthcare Treatment Manual* ([Consent Manual](#)).

4.1. Consent to use organs and tissue removed during medical, dental or surgical treatment

4.1.1. Consent by a person with capacity

The usual consent/request for medical treatment form must first be completed by the health practitioner (or their delegate) who will be performing the removal of the organ and tissue.

The written consent for the use of the organ and tissue (apart from the diagnostic purpose associated with the removal) must specify the purpose for and any conditions under which the organ and tissue is to be used. For example, the patient is to be given a description of how their organ and tissue might be used, such as for scientific and medical research, biobanking, teaching or study.

The *Human Tissue Act 1983* (NSW) allows consent to be general. Unless otherwise stated, the patient is to be informed that the term “therapeutic, medical and scientific purposes” has wide meaning and allows organs and tissue to be used for a large variety of purposes.

Where a patient places limitations on their consent, the organ and tissue must not be used outside the scope of the consent and the person’s limitations must be recorded.

The patient must be informed that consent to the use of organs and tissue is separate from consent to treatment and that treatment procedures are in no way affected by a decision not to consent to donation and use of the organ and tissue.

4.1.2. Consent from a person who lacks capacity

Section 21Z of the *Human Tissue Act 1983* (NSW) provides that where the patient lacks capacity to consent, the person responsible who is consenting to the medical, surgical or dental treatment may authorise, in writing, the subsequent use of the tissue removed during the procedure for therapeutic, medical or scientific purposes.

The NSW Health Policy and Procedure Manual *Consent to Medical and Healthcare Treatment Manual* ([Consent Manual](#)) sets out the requirements for establishing a person’s ability to provide substitute consent as the person responsible under Section 33A of *Guardianship Act 1987* (NSW).

4.1.3. Consent from a child

A senior available next of kin may authorise, in writing, the use of organs and tissue removed from a child during medical, dental or surgical treatment, for therapeutic, medical or scientific purposes (see Section 21Y of the *Human Tissue Act 1983* [NSW]). This does not include use for transplantation.

However, a senior available next of kin must not grant an authority if it appears, after making reasonable inquiries, that the child objects to the use of the organs and tissue for the purposes to be authorised or if there is another next of kin of the same or higher order who objects to the use of the tissue for the purposes to be authorised (Section 21Y(2) of the *Human Tissue Act 1983* [NSW]).

Where the child is considered to be a Mature Minor or *Gillick* competent, they may be able to give valid consent themselves. This must be done in accordance with the consent procedures outlined in the NSW Health Policy and Procedure Manual *Consent to Medical and Healthcare Treatment Manual* ([Consent Manual](#)).

NSW Health is committed to upholding the Child Safe Standards, including ensuring that:

- i) patients under 18 are supported and encouraged to participate in the consent process where appropriate, as outlined in the *Consent to Medical and Healthcare Treatment Manual* ([Consent Manual](#)).
- ii) consent processes for patients under 18 years are culturally safe, inclusive, and respectful of diverse needs. Information provided during the consent process must be accessible and tailored to the diverse needs of the child or young person, including their age, development, maturity, understanding and abilities.

Non-regenerative tissue must not be removed from a living child unless removed during the course of treatment carried out for the benefit of the child.

4.1.4. Consent for other therapeutic, scientific or medical purposes (including research purposes)

Where organs and tissue are removed from a living person as part of a medical, dental or surgical procedure:

1. Written consent for use of the organ and tissue for research must be obtained (either before or after the removal) from the person whilst alive (or parent/guardian if they are a child).
2. Consent must be sought in accordance with the research protocol as approved by a Human Research Ethics Committee.

For specific consent requirements, refer to the NSW Health Guideline *Use of Human Tissue for Research* ([GL2023_008](#)).

4.1.5. Where specific consent is not required

Section 34 of the *Human Tissue Act 1983* (NSW) provides for very limited circumstances for the retention and use of tissue, which have been lawfully removed from a living person, without consent.

It allows small samples of tissue, lawfully removed during medical, dental or surgical treatment, to be used without consent for purposes such as testing as part of a quality assurance/quality control program, audit or evaluation or pathology samples retained which are necessary for the accreditation of a hospital, forensic institution, laboratory or research institution.

Specific consent is not required to retain tissue in the form of a tissue slide or tissue block which enables microscopic examination of the tissue.

4.1.6. Consent for use of organs and tissue removed from a living person who subsequently dies

Where a person was living when the organs and tissue was removed during medical, dental or surgical treatment and subsequently dies, only a senior available next of kin (or their delegate) can consent to the use of the deceased person's tissue.

A senior available next of kin (or their delegate) must be satisfied that the deceased person had not, when living, expressed an objection to the use of tissue removed from their body, or that another next of kin objects (see Section 21ZA(2) *Human Tissue Act 1983* [NSW]). Consent must be recorded using the State Form *Consent and Authority for the Retention and Use of Tissue Removed or Expelled During Treatment of a Now Deceased Patient* (SMR020034).

Section 34(1)(b2) of the *Human Tissue Act 1983* (NSW) and Clause 8(2) of the *Human Tissue Regulation 2020* (NSW) allows for tissue removed during medical, dental or surgical treatment to be retained for up to 72 hours in order to obtain consent. This may arise where the person was an emergency patient who did not have the opportunity to consent before treatment and died during the course of their treatment.

4.2. Procedure for obtaining consent for the donation of organ and tissue from a living person for transplantation to another person

4.2.1. Mandatory 24 hour cooling off period

The removal of organs and tissue for transplantation to another person cannot take place until 24 hours after the written consent was given by the living donor. This does not apply where the organ and tissue is being removed for treatment purposes (for example, the femoral head from a total hip replacement) and the transplant of the tissue is a secondary outcome from the treatment of the living donor.

4.2.2. Removal of organs and tissue from a living adult

A medical practitioner who is not involved in the removal of the organ and tissue must certify (using the State Form *Human Tissue Act 1983 Consent and Certification for the Donation of Tissue by a Living Adult* [SMR020035]) that:

- the nature and effect of the organ and tissue removal were explained before consent was given

- written consent was given in the presence of the medical practitioner.

4.2.3. Use of non-regenerative tissue from an adult

Non-regenerative tissue refers to all tissue other than regenerative tissue (refer to [Section 1.2](#)).

Apart from removal in the course of treatment carried out for the benefit of the patient (see [Section 4.1](#)), non-regenerative tissue must only be removed from the body of an adult person for transplantation purposes.

Refer to NSW Health Policy Directive *Living Kidney Donation and Transplantation* ([PD2022_036](#)) for specific requirements regarding living kidney donation.

4.2.4. Removal of tissue from a living child

Consent for the removal from a child of regenerative tissue can only be given for the purpose of its transplantation to the body of a parent (being a biological parent, step-parent or adoptive parent), brother or sister of the child (see Section 10 *Human Tissue Act 1983* [NSW]).

If regenerative tissue is to be removed from a child for the purpose of transplantation, then the medical practitioner must certify (using the State Form *Human Tissue Act 1983 Consent and Certification for the Donation of Regenerative Tissue by a Living Child* [SMR020036]) that the:

- medical practitioner explained to the parent and to the child, before the consent was given, the nature and effect of the removal from the child's body of the tissue specified in the consent and the intended effect of its proposed transplantation
- parent and the child each understood the nature and effect of the removal of the tissue and the intended effect of its proposed transplantation
- medical practitioner is satisfied that the child or the parent and child were of sound mind
- the parent and the child each understood the nature and effect of removal of the tissue and the intended effect of its proposed transplantation
- the consent was freely given, and
- child remains in agreement with the proposed tissue removal and transplantation.

Where the child is considered to be a Mature Minor or *Gillick* competent, refer to the consent procedures outlined in the NSW Health Policy and Procedure Manual *Consent to Medical and Healthcare Treatment Manual* ([Consent Manual](#)).

Use of regenerative tissue from a child not capable of understanding the procedure

Where a child is not capable of understanding by reason of their age, the general nature and effect of the removal of tissue, a medical practitioner's certificate must be issued under Section 11A of the *Human Tissue Act 1983* (NSW). In this case, only transplantation to the body of a brother or sister is allowed.

In this case, a medical practitioner must certify that:

1. The medical practitioner explained to the parent the nature and effect of the removal of tissue from the child's body and the intended effect of the proposed transplantation.
2. Parental consent was given in the presence of the medical practitioner.
3. At the time of the consent, the medical practitioner is satisfied that the parent was of sound mind, understood the nature and effect of removal of the tissue, and that consent was freely given.
4. The medical practitioner is of the opinion that the pre-conditions for child tissue donation (in this setting) are satisfied:
 - the child, by reason of his or her age, is not capable of understanding the nature and effect of the removal of the tissue and the intended effect of its proposed donation
 - the brother or sister of the child is likely to die or suffer serious and irreversible damage to his or her health unless the tissue to be removed from the child is used in their treatment of that brother or sister
 - any risk to the child's health (including psychological or emotional health) caused by removal of tissue is minimal.

The above certification is effective only if a second medical practitioner, who is a specialist in paediatric medicine or paediatric transplantation, certifies that:

1. They are of the opinion that the pre-conditions for child tissue donation are met.
2. They are acting as an independent medical practitioner, meaning their primary role in providing this opinion is to ensure the health of the child from whom tissue is being removed.
3. They are not responsible for care of the sibling in whose treatment the tissue is to be used.

The Family Court of Australia has power to authorise the removal of tissue from a child incapable of consenting by reason of their age. In considering any such application, the Family Court must ensure that the interests of the child are paramount. In the case *Re Inaya (Special Medical Procedure)* [2007] FamCA 658, the Family Court authorised donation to a first cousin. The Family Court is not subject to the limitations set by the *Human Tissue Act 1983* (NSW).

It must be noted that the Family Court may make orders with respect to donation of regenerative tissue by minors that may override parental and/or medical consensus in some circumstances.

Local health districts are to notify the Executive Director, Legal and Regulatory Services & General Counsel of the Ministry of Health of any potential applications to the Family Court as outlined in the NSW Health Policy Directive *Significant Legal Matters and Management of Legal Services* ([PD2017_003](#)).

Local health districts that carry out bone marrow transplantation must ensure that the provisions in the *Human Tissue Act 1983* (NSW) or the orders of the Family Court are followed.

If there is to be a difference of opinion between the regenerative tissue transplantation specialist and the independent assessor of the patient, a second independent assessment of the donor should take place at another campus.

Use of non-regenerative tissue from a child

Non-regenerative tissue must not be removed from a living child unless removed during the course of treatment carried out for the benefit of the child.

4.2.5. Revocation of consent

If the donor indicates that they wish to revoke their consent or in the case of a child, if the child no longer agrees with removal and transplantation of tissue, the designated officer for the hospital must be informed and act on this. The revocation of consent must be recorded in the donor's medical record. For further information on the role of the designated officer, see the NSW Health Policy Directive *Designated Officer* ([PD2025_040](#)).

4.3. Restrictions on the use of tissue from children in the care of the State

Where tissue was removed from a living child who subsequently dies and the child immediately before their death was in care of the State, the tissue must not be used for any medical, therapeutic or scientific purposes other than donation for transplantation.

5. Privacy

If the purpose for which the tissue has been collected requires the collection, storage, linkage or potential disclosure of the person's personal health information, disclosing this to the donor is an important part of the consent process and must be acknowledged in the consent documentation.

In accordance with Section 37(2) of the *Human Tissue Act 1983* (NSW) and the *Records and Information Privacy Act 2002* (NSW), NSW Health employees are prohibited from disclosing any information which could lead to the identity of an organ and tissue donor (whether deceased or living) or recipient becoming publicly known unless authorised under the *Human Tissue Act 1983* (NSW).

For current requirements in the management of personal health information refer to the [Health Records and Information Privacy Act 2002](#) (NSW) and NSW Health Policy and Procedure Manual *Privacy Manual for Health Information* ([Privacy Manual](#)).

6. Disposal or return of organs and tissue

Information must be provided at the time of obtaining consent as to how the organs and tissue may be disposed of after the therapeutic, medical or scientific use has expired.

Legally there is no ownership of excised organs and tissue. However, if the conditions outlined below (see Section 6.1) are met, there may be no objection to the patient or senior

available next of kin taking home organs and tissue removed or expelled from the patient's body while they are in hospital.

A responsible medical practitioner or health professional must be satisfied that the arrangements for returning the organ and tissue to the patient/senior available next of kin and the eventual method of retention or disposal do not present a public health risk.

This requires an assessment by the local health district/specialty health network of the request and the provision of additional information to be given to the patient/senior available next of kin on the public health requirements described below. The State Form *Authorisation for the Release of Human Tissue to a Patient or Senior Available Next of Kin* (SMR020033) must be completed.

Tissue returned to the patient/senior available next of kin or their delegate for appropriate storage or disposal must be triple packed as required by the National Pathology Accreditation Advisory Council's *Guidelines for Approved Pathology Collection Centres* (Third Edition, 2013) ([National Pathology Guidelines](#)).

If a patient/senior available next of kin does not consent to their tissue being used for other medical, therapeutic or scientific purposes and does not request its return, the tissue must be disposed of in accordance with the hospital's standard waste management procedures.

Where organs and tissue are removed and subsequently deemed not suitable for transplantation, the retrieving surgeon or delegate must notify the Donation Specialist Coordinator. The organs and tissue may be utilised for research purposes (if consent was granted) or respectfully disposed as per standard hospital protocols. Organs must be returned to the body when requested by the coroner.

6.1. Assessment of requests

Requests for the return of organs and tissue to the patient/senior available next of kin for disposal are to be assessed by the relevant senior medical or health care professional caring for the patient.

'Return to the body' will not be offered as part of the routine consenting process. However, specific senior available next of kin requests that organs not transplanted be returned to the body must be considered.

Any request made for the release of fetal tissue must be referred for assessment in conjunction with a social worker or counsellor.

NSW Health staff must discuss the request with the patient/senior available next of kin including:

- the grounds for the request
- the proposed storage and/or final disposal method for the material (which must be in keeping with the requirements of this Policy Directive)
- whether the intended disposal method constitutes a public health or other safety risk (see [Section 6.2](#)).

The patient/senior available next of kin must be informed of the requirements of this Policy Directive and given the opportunity to clarify the requirements.

It must be made clear to the patient/senior available next of kin, in an appropriate and sensitive manner, that Section 32 of the *Human Tissue Act 1983* (NSW) makes it an offence to sell or supply (or offer to sell or supply) human organs and tissue to another person for anything of valuable consideration.

6.2. Public health and safety requirements

Human organs and tissue cannot be released to a patient or senior available next of kin where:

- it may be contaminated with cytotoxic, chemical or radioactive waste
- it is reasonably believed to be infected with a prescribed infectious disease as defined in Clause 79 of the *Public Health Regulation 2022* (NSW).

Prior to disposal or transfer to another private storage arrangement, the organ and tissue must not to be removed from the container in which it is released.

Disposal or other appropriate storage must take place within 8 hours of the organ and tissue being removed from the hospital. Refer to NSW Health's [Disposal of the deceased](#) for further information.

6.3. Documentation

Appropriately detailed documentation of the assessment and discussion with the patient/senior available next of kin must be made in the patient's medical record.

If a decision is made to release the human organs and tissue for disposal, the State Form *Authorisation for the Release of Human Tissue to a Patient or Senior Available Next of Kin* (SMR020033) is to be completed by the patient/senior available next of kin who is taking possession of the organs and tissue. The original form is to be filed in the patient's medical record. A copy of the form and a letter certifying that the person is travelling with human tissue in their possession (see [Appendix 1](#)) must accompany the person with the tissue.

It must be clear to the person who receives the organs and tissue that they are responsible for the safe and secure storage of the transferred organ and tissue.

If the organ and tissue to be returned is from a deceased person, the senior available next of kin is to be asked to provide copies of appropriate identification for documentation.

6.4. Minimum requirements for the preparation and release of organs and tissue

The medical practitioner who is preparing the organs and tissue for release for private disposal must have regard to their hospital's local protocol on the handling, storage and preparation of human organs and tissue for transportation.

The organ and tissue may be immersed in formalin but must be drained and must not be released in formalin. It is important in some cultures that placental tissue is not placed in formalin.

The National Pathology Accreditation Advisory Council's *Requirements for the Packaging and Transport of Pathology Specimens and Associated Materials* ([Packaging and Transport Guidelines](#)) recommend triple packaging for the transport of all human organs and tissue.

A label identifying the organ and tissue will need to be affixed to the container. The outer container must be marked "tissue for collection by" and include the name of patient/senior available next of kin/mother (for fetal tissue under 20 weeks gestation) and the date of the procedure/miscarriage/death.

The vessel containing the organ and tissue is to be stored in a refrigerated unit by the organisation pending collection by the patient/senior available next of kin/other delegated person who will collect the tissue.

If the organ and tissue is to be transported by the patient/senior available next of kin, it is to be placed on ice in a waterproof container when released to the patient/senior available next of kin. Dry ice cannot be used. Staff must inform the patient/senior available next of kin that refrigerated transport must be used where the journey is more than 8 hours.

6.5. Burial or cremation of organs and tissue

Some families may choose to arrange their own cremation or burial of organs and tissue, including fetal tissue (less than 20 weeks gestation), which is at their own expense. Fees for cremation of individual organs and tissue may be expensive and families are to be encouraged to check these costs before making a decision.

Crematoriums require a letter from a medical officer certifying that the organs and tissue is the 'tissue/limb/etc. of patient x'. A letter will also be required from the patient/senior available next of kin stating that they request cremation of the organ and tissue (including, but not limited to, a limb).

In the case of fetal tissue, crematoriums require a letter from a medical officer certifying that the tissue is the fetal tissue and noting the gestational age.

The patient/senior available next of kin will need to organise transport of the appropriately packaged tissue to the crematorium.

6.6. Burial of a body on private land

In addition to the public health requirements set out above, burial of a body on private land or in a garden requires council consideration. Further information is available at NSW Health's [Disposal of the deceased](#).

7. Appendix

7.1. Appendix 1: Example of letter to be issued to person travelling with human tissue in their possession (on facility letterhead)

To whom it may concern,

This is to certify that _____
(Name of person authorised to travel with human tissue in their possession)
is travelling with human tissue in their possession.

The tissue is sealed inside a container and there is no risk associated with transporting the tissue stored in this manner.

Person certifying the packaging of the tissue:

Name: _____

Designation: _____

Institution/Hospital: _____

Contact: _____

Signature of authorising person: _____ Date: ____/____/____